

Apollo Hospitals Enterprise Ltd. (APOLLOHOSP)

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Transcript of AHEL Q 2 FY26 Earnings Call Page 1 of 16
Apollo Hospitals Enterprise Limited
Transcript of Q 2 & H1 FY26 Earnings Conference Call
November 7, 2025

Moderator: Ladies and gentlemen, good day, and welcome to Apollo Hospital Limited earnings conference call. As a reminder, all the participants' lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Devrishi Singh from CDR India. Thank you, and over to you, sir.

Devrishi Singh: Good morning, everyone, and thank you for joining us on this call, hosted by Apollo Hospitals to discuss the financial results for Q2 and H1 FY26, which were announced yesterday.

We have with us today the senior management team represented by Mrs. Suneeta Reddy – Managing Director; Mr. A. Krishnan – Group CFO; Dr. Madhu Sasidhar – President and CEO of the Hospitals Division; Mr. Madhivanan Balakrishnan – CEO of Apollo HealthCo; Mr. Sriram Iyer – CEO of AHLL; Mr. Sanjiv Gupta – CFO of Apollo HealthCo ; and Mr. Obul Reddy, CFO of the Pharmacy business.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward -looking in nature and may involve risk and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide number 2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been calculated earlier, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy: Good afternoon, everyone, and thank you for taking time to join our earnings call. I believe that you have received the earnings documents, which we shared yesterday.

Consolidated revenue was at INR 6,304 crore , a 13% year -on-year growth. Consolidated EBITDA was at INR 941 crore , registering an increase of 15% year -on-year. Within this, the Healthcare Services EBITDA was at INR 781 crore , registering a growth of 8% year -on-year and Healthcare Services margin remained robust at 24.6%.

We are pleased to talk about the sustained and strong momentum from quarter 1 into quarter 2, delivering strong growth across all three verticals, Healthcare Services, Apollo HealthCo and AHLL. Despite the seasonal impact from the onset festival period, focussed execution has enabled us to report broad -based growth and resilient operating metrics.

Transcript of AHEL Q 2 FY26 Earnings Call Page 2 of 16

Healthcare Services business has delivered a 9% year -on-year revenue growth to INR 3,169 crore . The revenue growth was driven by insurance and cash patients together accounting for 86% of the inpatient hospital revenue.

Q2 FY25 had a higher incidence of seasonal medical admissions, leading to a high base.

Medical admissions were lower in Q2 FY26. The low growth in medical admission was offset by a 14% increase in revenue from CONGO Speciality.

The reduction in number of patients from Bangladesh has had a 1% impact on Healthcare Services revenue in Q2 FY26.

Surgical volumes grew by 3%, supported by our continued focus on CONGO specialty.

Cardiac, oncology, neurosciences, gastro and orthopaedics - which remain a key growth engine , delivering a 6% volume and 14% revenue growth on a year -on-year basis. Group -wide occupancy stood at 69% in Q2 FY26.

As shared earlier

, we have withdrawn the ARPOB metric and introduced the average revenue per patient , ARPP as a more accurate measure of realization. ARPP in Q2 FY26 was 173,380 , recording a growth of 9% , through a combination of better clinical mix and regular tariff increased with inflation.

Within the Healthcare Services business, we have delivered a ROCE of 30.3% with a balanced ROCE across all geographies, Metro Tier 1 and Tier 2 .

Moving ahead, Apollo HealthCo reported revenues of INR 2,661 crore marking a 17% year -

on-year growth. Apollo Health & Lifestyle revenues also increased by 17% to INR 474 crore during the quarter.

Private label and generics were at 15.2% of total pharmacy revenue. Our digital platform, 24/7 added 3 million new users, taking the total to 44 million users. The platform GMD was at INR 723 crore, growing 16% over the same period in the previous year.

The pharmacy distribution business in Apollo HealthCo reported an EBITDA of INR 181 crore as against INR 153 crore, higher by 19%. Losses in the digital vertical was at INR 71 crore compared to INR 101 crore in the same quarter last year. As a result, Apollo HealthCo reported an EBITDA of INR 110 crore in Q2 FY26, up from INR 52 crore in Q2 FY25.

AHLL delivered an EBITDA of INR 50 crore, representing a 21% year-on-year growth, with margins improving to 11% from 10% in quarter 2.

During the quarter, the Chief Minister of Delhi inaugurated Apollo Athenaa, Asia's first dedicated cancer center for women in Defense Colony. The facility is designed to tackle the growing burden of breast and gynaecological cancers and integrates precision medicine survivorship programs and world-class treatment protocol. We also performed a soft launch of the Multispecialty Tertiary Care hospital in Pune. We continue to make steady progress on our other expansion projects across key metros including Kolkata, Hyderabad, Bangalore and Gurugram with commissioning lined up in the upcoming 12 months.

Transcript of AHLL Q 2 FY26 Earnings Call Page 3 of 16

The statutory processes for the restructuring within Apollo HealthCo and the Keimed merger continue to proceed as per plan, and we remain well positioned to capture the full benefit of scale within Apollo HealthCo.

Half year H1 FY26 Results : The consolidated revenue for H1 FY26 stood at INR 12,146 crore, a growth of 14% on a year-on-year basis, supported by balanced expansion across all three verticals. Healthcare Services revenue at INR 6,104 crore, up 10% year-on-year, driven by continued traction in high-end specialty and improving clinical mix. Apollo HealthCo delivered revenues of INR 5,132 crore, registering an 18% increase year-on-year. While AHLL grew 18% to INR 909 crore.

Consolidated EBITDA for the first half was INR 1,793 crore, reflecting a 20% increase year-on-year. PAT stood at INR 910 crore, up by 33%. This is a strong half year performance, which reinforces our ability to sustain growth momentum while maintaining financial discipline and operating efficiency.

Let me close by saying that our quarter 2 and H1 performance underscores to continue its strength of Apollo's integrated health care ecosystem and our commitment to delivering clinical excellence at scale. We remain focused on deepening our leadership in core healthcare services, accelerating digital in retail health and expanding our network through strategic capacity additions across key markets.

With a disciplined approach to execution and a clear road map for growth, we are confident of enhancing our performance in the coming quarters, and we continue to create long-term value for patients, partners and shareholders.

On that note, I would like to hand it over to the moderator and open the line for questions, I have Krishnan - o

ur Group CFO; Mr. Madhu Sasidhar - CEO of Hospitals Division; Madhivanan - CEO of Apollo HealthCo; Sriram Iyer - CEO of AHLL, Obul Reddy and Sanjeev from Apollo HealthCo, we will take all of your questions. Thank you.

Moderator: We will now begin with the question-and-answer session. The first question is from the line of Binay Singh from Morgan Stanley.

Binay Singh: My first question is how to think about organic growth for hospitals. Keeping in light two points; - first is that the Bangladesh impact incrementally will go away, so that will be 1% positive.

Secondly, there is lots of news on insurance pricing that hospitals are planning to keep steady for next year. So, keeping both these, could you comment on this and then guide on organic hospital growth for next year?

Suneeta Reddy: Yes. I think we are quite confident that we will get back into 30%. We say this because Bangladesh, at least 60% has started coming back in October and we believe that we will mitigate the impact of losing one territory. Also, we are exploring new markets, including the Northern markets in Uzbekistan, etcetera. So those that are surrounding markets, Africa, some with Indonesia, Iraq. New markets added to Bangladesh coming back will definitely give us an upward trajectory.

With regard to insurance, we continue to stay in dialogue with the insurance companies. And I think that there is a very good way forward because all of them represent different parts of the value chain, and we recognize how important it is for us to cooperate and work together.

A. Krishnan : And on the pricing front on insurance also, I am not sure about this commitment that you are talking of. Typically, as you know, in any case, our insurance contracts are once in two years. So, by that definition, itself we do not get a price increase every year. So, what happens is every two years, it gets reset. So, if you reset the contract this year, we, it is the same pricing applies for two years, while our inflation is always annual, which you know. So that is the way it has been working, and it is the same way that it continues to be. Certain contracts will come up for renewal this year, certain others will come up for renewal the next year. So, it is a rolling contract renewal that works across the system. It continues the same way.

Madhu Sasidhar: And Binay, if I can add, even within this quarter, you will see that there is, I think Ms. Suneeta spoke about it a significant improvement in the quality of the revenues. There has been an increase in the complexity of cases with substantial improvement in our CONGO specialties and high complexity cases. We have also invested a lot in the past two quarters in building solid programs and heavily into recruitment, especially in our mature units. I think these will all add to that organic growth that you were asking.

Binay Singh: Thanks for that detailed answer. Secondly, just one question on the presentation on Slide 16, where we talk about capacity expansion. Earlier we used to say FY26 commissioning number. Now we are putting it under FY '26-'27. So, is there any change in ramp-up plans just to check that for the next 2 years?

A. Krishnan : So, the way we are looking at it is this. Today, if you are looking at, and I will comprehensively handle this slide because there are a couple of questions also which will come up. One was six new hospitals is what we are looking at adding in the next year, this year to next year. This coming quarter, you would see us already add, started commissioning, we already soft commissioned the Defense colony Cancer Hospital. We have also soft commissioned Pune. So, you will see that Pune and Defense Colony, both we will start report

ing numbers from this quarter itself which is Q3.

Come to Q4, we will be starting the Sarjapur Bangalore Hospital as well as Calcutta. Come to end of Q4 or early Q1 is when we are looking at Hyderabad because there in Hyderabad, as you would have seen, the costs have gone up by INR 35 crore because we have now decided to add a comprehensive oncology program also there, which includes the radiation therapy equipment with results in us, and that will require some more time to build. So, which is why it will be more around Q1.

Even in Gurugram, it will be more around Q1 because we have enhanced the overall facilities and doing some more of private rooms, etcetera, in line with what the market demands. So, the way you should look at it is Q3, Q4, Q1. In that order, we will be looking at starting all these six hospitals. The balance brownfield, which is also something that we have now added. We have also started work on Jubilee Hills and Secunderabad now. So, all those brownfield

expansions will also come next year, mid of next year, etcetera, mid - to end of next year.

Suneeta Reddy: And in Bangalore, Malleswaram and Mysore expansion, work has started on both. So, you should see it happening. Next year, we will open out those beds.

A. Krishnan : So, by end of next year, all the census beds that you are seeing should be fully operational. Of course, as you know, we start 50% as we start and then keep ramping up. So next year is going to be quite a busy year for us. And you will see all of these fully commissioned by end of next fiscal.

Binay Singh: Thanks team, very clear.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My first question is again on hospitals. So, between now and end of FY27, as you indicated, six hospitals coming up. So, I understand '26 Pune will be key addition and the majority will be coming in '27. So, if you can comment a bit on the impact on EBITDA margin trajectory due to the new costs coming up?

A. Krishnan : So, Pune and Defense Colony is this quarter, Sarjapur and Calcutta is also Q4. So, from an FY '26 perspective, FY perspective, these four hospitals come in FY26 itself. And Hyderabad and Gurugram will come into the next year. We continue to believe that next year, overall EBITDA losses from these hospitals should be around the INR 150 crore number, which is what would be the EBITDA losses from these hospitals, but we will come back to you closer to the Q3, Q4 to once we commission some of these hospitals, but we do not expect it to be higher than that.

Damayanti Kerai: And these losses include fully build-up costs, right? Once you commission a unit, most of your doctor cost, your facility costs, etcetera, are already in place.

A. Krishnan : Yes, that is correct.

Damayanti Kerai: Okay. My second question is on your spend on Apollo 24/7. So, the last two quarters, I think somewhere INR 94, INR 96 crore kind of spend, which we have seen. So, what kind of headroom you have, to reduce from these levels? Or these are more sustainable cost levels to look at?

Madhivanan B: So Damayanti, when you say spend, it is a cost structure. So, a big chunk of the cost has been as an individual entity, we have reduced it quite a bit. So, this would be, in a way, a new normal.

But as we get into the program of aligning between the three entities, Keimed, Apollo Pharmacy, PD, and we will see a few more, a little bit more of synergies coming through as both the teams will merge. But our biggest expenses are primarily on the marketing side, so which has been, it is come to a very rational level.

So, you will see some more, but that will typically happen from the next financial year. Q3, Q4, more or less, we will

kill, there is nothing much more to bring down on the expense line. So, focusing on the revenue side of the story.

Transcript of AHIL Q 2 FY26 Earnings Call Page 6 of 16

Damayanti Kerai: And the target of achieving cost breakeven for 24/7 by end of this fiscal remains or we can...

Madhivanan B: We are on course. There might be one hiccup in the form because we are investing reasonably strongly on the insurance side of the business. We are seeing some very good traction. We have completed some of the integration with the health side, we are exploring the life. So, there might be a little bit of a hiccup here or there, but we are on course, as we speak.

Damayanti Kerai: That is helpful.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar M: Again, just on Apollo 24/7, on the GMV side. Probably 7,200, 7,300, sort of a stable for the last 3, 4 quarters while you already highlighted in terms of how the cost measures will help in getting the better profitability. If you could share your thoughts on, how do we think about improving the GMV?

Madhivanan B: So you have to look at the GMV from three perspectives. One, how is the pharmacy business growing. Within the pharmacy business itself, there were two levers. One, what we call as a platform revenue, which is primarily driven by Apollo 24/7 the app and the website, which has been growing at a very good pace of around 30% on a year-on-year basis. And on a quarter-on-quarter basis, we are in the range of around 5% to 7%.

In this quarter, we actually exited out of a few B2B businesses, because we were considering to walk out of them because of some of the bottom line related and our quest of trying to break even. So, this GMV reduction. That is why you are seeing that the 16% growth, that being primarily from the pharmacy side. So, that was one reason.

And the third was the GST, because earlier we used to report the total numbers on a total GST basis. Given the advantage that we have received as an industry, it has shaved off around 6% from the top line. It does not impact us on the bottom-line side. So that is the only reason. So, you will start seeing the increase on a quarter-on-quarter basis because now it is a new normal.

Tushar M: Got it. In terms of any other, let us say, business aspect, if you can, so the GST is more like a transitional period probably, but any other aspect in terms of, to drive the GMV?

Madhivanan B: No we will continue to grow what we have our original guidance of growing between 25% to 30% on the overall. Our diagnostic business is picking up, in fact, we buck seasonality. So, we will be back on course again, on that side of the story. On the hospital side of the business, the business that we try to drive on the consults, we are relooking at our approach so that we can become much more relevant in the overall scheme of things as far as the hospital is concerned.

And the last one, the insurance will, like I told you, will take a little bit of time, but we have started clocking in some good numbers with just two cities, which is the NCR and Hyderabad. You will start seeing the new businesses picking up from Q4 onwards.

Tushar M: So just on the hospital side, the Karnataka cluster, I have seen IP volume decline . If you could

Transcript of AHEL Q 2 FY26 Earnings Call Page 7 of 16

share. ...

A. Krishnan : So, Karnataka region, there was a drop in the medical admission , significantly in that region , particularly. If you look at the drop overall of the 6% that we reported in this region, there is a medical volume drop of 15%, while surgical volume went up by 2% and cat

h also went up by

13%. So, I think it is really the seasonal medical admissions that we see in Q2 in Karnataka , we saw in Q2 of last year . Definitely was not there this year.

But otherwise, this is why if you look at the average revenue per patient in this region , it is gone up by 14% because the balance cases, which is surgical and cath, that makes us a higher ARPP compared to the ARPP of medical admissions. So clearly, that is the reason. So, the core continues to remain intact.

Madhu Sasidhar: Yes, that is correct. And if you trace those medical admissions back to last year, it was almost entirely a very bad dengue season, which we have not seen this year. So, I think it is a year - to-year seasonality , meaning volume that we do, especially in the high complex impact it continues to be strong. I think that growth will continue into this quarter.

Tushar M: That is helpful , thank you.

Moderator: The next question is from the line of Harith Ahamed from Aventus Spark.

Harith Ahamed: My first question is on margins for Keimed . We have seen some softness there of around 30 to 40 basis points decline on a Y -o-Y basis. In the past, you have talked about some restructuring and buyout of minority stakes, et cetera, within Keimed. Is that activity over? And how should we think about Keimed margins and especially in the context of the overall margin guidance of 7% by 4Q next year.

Sanjiv Gupta: Yes, you are right in saying this Q2, we had slightly drop in the EBITDA margins for Keimed , but this is only onetime integration and scheme related expenses, which got accounted in Q2. So, you did not see the same happening in , from next quarter onwards. And as suggested earlier also, over a period of time , you are looking at 20 to 30 basis points over and above 3.1%, which normally used to be Keimed EBITDA since last two, three quarters to be happening as we move forward.

As far as the overall 7%, which I think you are referring to the guidance of Q4 FY27, wherein we are suggesting about INR 25,000 crore of revenue run rate, with a 7% EBITDA. I think we are hopeful that we should be able to hit that mark. And progress is there quarter -on-quarter. This quarter, if I look at the H1 of FY26, we are at about INR 9,200 crore of turnover, which makes it to about INR 18,000 crore plus and EBITDA is 4.4% I think, once we get into breakeven, the margin should come closer to 7%.

If you exclude the digital losses, we are already at about 6.2%. And as Mr. Madhivanan suggested that somewhere in Q4 we will be very near to breakeven. So, I think after that, you would start seeing overall EBITDA also to be in the range of 6% plus. And one year forward, we should be in the range of about 7%.

Transcript of AHEL Q 2 FY26 Earnings Call Page 8 of 16

Harith Ahamed: Got it. My second question is on Specialty Care segment within AHLL. Growth has been a bit soft there and you called out competitive headwinds in the segment. So, trying to understand which verticals within Specialty Care we are seeing higher competition and what is the outlook here.

Moderator: The line for Mr. Sriram got disconnected .

Suneeta Reddy: Okay , could you repeat the question, we will take it.

Harith Ahamed: So I was asking about the Specialty Care segment within AHLL. We see a relatively modest growth there for that segment in this quarter. And in the presentation, you have called out competitive headwinds for the segment. So within , I understand there is multiple verticals like Cradle and Spectra within Specialty Care. So, trying to understand , which vertical we are seeing greater competition ? And what is the outlook here?

Suneeta Reddy: So, in terms of competition, clearly, the only one that has serious competition is Diagnostics because Spectra there is

no competition. And in Cradle , it is only where our Cradles are present , there is very little competition, except in Karnataka where Cloudnine, has a big market share. Having said that, I think that our focus will be on primary care. It will continue to be focused on growing diagnostics as well as growing the clinics, which is really primary care. Creating clinics with GP s at the center , to be able to look after communities and act as a funnel to Apollo Hospitals. So, this is what we are currently focusing on. And of course, dialysis continues to d well.

Harith Ahamed: Got it, madam. Thank you.

Moderator: The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Ma'am I think you mentioned that you expect growth to get back to 13% for the Healthcare Services business. This I assume would be organic growth and the expansion should add to this growth? Would that be a fair assumption?

Suneeta Reddy: I think over a three -year period, you will see that , there is headroom for growth within the system. This should result in 13% growth in the existing beds and an additional 5% coming from new b eds in the next 36 months.

Neha Manpuria: So, 5% will get added over the next three years. Would that be right?

Suneeta Reddy: Next two years, you will see another 5% coming.

Neha Manpuria: Okay , got it. And on the GMV growth in the digital business, could we get a breakup of what would be the split from GMV in pharmacy and insurance , new businesses? And how should we see this let us say, when the business achieves breakeven, what would be the mix of that GMV? Just trying to understand how the revenue and margin should move based on this mix?

Madhivanan B: Let me not give the exact breakup, but typically, the pharmacy business constitutes the biggest

Transcript of AHEL Q 2 FY26 Earnings Call Page 9 of 16

chunk out of this. And this is the one which has a direct impact on our profitability, because the unit economics is what drives this breakeven that we are working towards. So, any kind of a growth in GMV, which is on a profitable basis at a unit economics level, straight away contributes a breakeven. So, the way to read it , would be, that what percentage of our total GMV is being contributed through the e -pharmacy business . That is layer number 1.

The diagnostics business is again directly linked to GMV, albeit the numbers are not as large as that because the margin structure is reasonably better. So that constitutes the second portion. The third layer of business, which is the hospital business , is predominantly fee driven. We are more a tech services and a lead operator for the hospitals, being driven through the digital mechanism. So that is more or less independent of the GMV growth.

As far as insurance is concerned, this is more of the GMV equivalent in the financial services industry is gross written premium which is what we log in. So here again, because we are approaching a model wherein, we want to make insurance as affordable as possible. We are actually promoting a more EMI driven kind of a business in this model.

So here again, we will start reporting the collected premium into our numbers. So, these are the four layers . E-pharmacy the biggest constituent. Diagnostics is the second big one. The hospital contribution of the GMV is primarily driven by fees and therefore, will not make such a big play. And the fourth one is the insurance business, which will come to the party in another maybe a quarter or so. And like I said, 55% to 60% of the to tal pool comes from the pharmacy.

Neha Manpuria: And from a profitability perspective, the increase , I mean, for us to improve margins to, let us say, the 20%, the pre -operating margin for 20%, 25% would essentially then depend on pharmacy bus

iness growing? Or would it dip, as insurance picks up scale, that would be a bigger driver of the margin ?

Madhivanan B: Pharmacy is the current driver of the business, that is a reasonably established business. But the insurance at this point of time is still on a breakeven mode. Once that gets in the break even move, it starts contributing to the profitability in a much more disproportionate sense. But to read it for the next two quarters, I would focus on the pharmacy business.

Neha Manpuria: Understood , thank you so much.

Madhivanan B: I just want to highlight one more point. As we speak, just to give some confidence back to how our path to profitability is growing along , all the three lines of businesses that I spoke about , pharmacy, diagnostics and consult business at a C M1 level , has turned positive , in each one of them at an individual level. Our immediate objective in the coming quarters is to drive the CM2 and then the CM3. Like I told you, maybe the insurance business will take a little bit more time, but we will break that also up the curve .

Moderator: The ne xt question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: I know you mentioned about the seasonality and the reason why the utilization fell from 73% overall to 69%, and I get it. But I am just trying to look from a forward path perspective, right .

ALOS has been consistently declining. This is obviously as we are able to be more efficient in

Transcript of AHEL Q 2 FY26 Earnings Call Page 10 of 16

terms of our processes and stuff. But is there a theoretical lower limit where we reach on ALOS? I think that is question one.

And my larger point being, when will we start seeing better volume growth? Because utilizations have been below 70 %. And is there a plan where at some point of time, we needed to go to 70 , above 70 at all or our business model does not need to be like that ? I am just trying to compare ourselves with some of our peers whose utilizations are higher.

Suneeta Reddy: So, 70% is definitely a benchmark that we are looking at. ALOS has dropped by 7%. So, the use of new technologies, whether it is the cardiac where we have minimally resistant or robotics. So, this is really driving down ALOS and allowing us to discharge patients much faster. With regards to occupancy, I think that 70% is definitely the target that we hope to reach. Having said that, we are looking at improving payer mix.

We are focusing on the corporate, which earlier, I think we had not put enough focus there.

We also focus on retail as well as international coming back to us. With some of these initiatives, I think that we will move closer to 70%. Having said that, our metro hospital s have actually crossed 70%. And in October itself, we are looking at higher occupancy coming from metro. We have a separate plan for the non-metros.

Madhu Sasidhar: So, it is a good question, and I will take the ALOS question first. As Mr. Suneeta said, the benefit s that you are seeing are the results of us putting in driving our digital transformation, putting in electronic command centers in each of our hospitals and reducing variability. And I think we are probably not going to see sustained reductions in ALOS other than a few of our units that are starting to catch up.

I think where this puts us is in a place where we can drive utilization higher. We can more easily take on much higher levels of occupancy with less variation and more predictability, because the barrier to driving occupancy to much higher in acute care hospitals is being for best when there is high variability day in patient admission.

I think our digital transformation allows us to address this more efficien tly. So clearly, we are targeting much higher levels of occupancy. And in some hospitals,

it is possible to drive it to

80% and higher, especially when we are targeting more elective and semi -elective.

Shyam Srinivasan: Very helpful. And just a second question on the EBITDA loss guidance that you shared for the six hospitals. I heard it is INR 150 crore . I think you called it off at INR 150 crore . So, I just want to understand does this hit us in the full fiscal '27? Is it spread around just from our past experience 2015 to 2018, where some of the bed additions at that time led to , like very volatile margin trajectory? Is there something that you are doing this time differently , to ensure even some of the financial metrics are a little bit smoother, if I say so.

A. Krishnan : Surely, we are, these are all in existing markets. We go all in existing markets. So, if you look at places like Delhi, Hyderabad and Calcutta etcetera. We have a very clear plan on how we can ramp up. And as we have said, our internal target is to get all of them breakeven in 12 months. And given that it is all going to be if you look at it, it is going to be first two hospitals in Q3, followed by another two in Q4 and then Q1, there is a kind of , it is being spaced out a bit.

Transcript of AHEL Q 2 FY26 Earnings Call Page 11 of 16

And yes, in the first half of next quarter, there could be a bit higher than , it will not be equally spread, but then first half would be a bit higher and then it will come off. But we will start showing that separately in our earnings presentation as we have done in the past as well, so that you get the confidence that we are in track to achieve those numbers that we have cited.

Suneeta Reddy: And I believe that it wo uld no t be a significant impact on EBITDA margin, because we have strong cash flow s, EBITDA is supposed to be (Inaudible 0:38:45) as we open new hospitals.

A. Krishnan : And as Dr. Madhu said, the existing hospitals also, we have room for growth, and we are working on that. So that also should start supporting in the next year on the volume as well as the ARPP driven growth. So, we are working on both.

Shyam Srinivasan: Got it , thank you and all the best.

Moderator: The n ext question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: One on the insurance business. How are you selling the policies now? Is it mostly online?

Madhivanan B: It is a combination of all the three. Our first attempt is to try and sell it digitally because we have no intentions of going and spending on marketing spends and trying to get new customers. So, we are primarily focusing on this INR 1 crore plus customer and a INR 44 million registered customers. So, it is predominantly driven by digital. However, insurance being such a complicated piece, a small ticket item sells very well on digital. However, the moment the ticket size goes into the range of INR 20,000 to INR 30,000 people seek out an assistance.

So, we are building a capability of a call center, which is right now 300-seater. We intend to take up to around 500, as a capacity building and the productivity norms work out. So, think of it as a combination and omni model of leads and some business through digital and the corresponding ones through a call center mechanism. We are not in the process of putting any kind of a manpower on the field. If at all any, it would be more from a services perspective. But that is not the channel that we are focusing on.

Bino Pathiparampil: And do you plan to promote it through your physical pharmacy stores at all?

Madhivanan B: Here again, we will come out with some specific products such as, let us say, a vector insurance or a personal accident, which will be done more under the POSP model, but that is expected only around the next financial year. We would like to get the basics right first and

then go into some of those models in which case maybe out of the 7,000, we might identify 1,000 -odd outlets without putting a person in the place, we are not intending to increase the cost, but we will work very closely with the APL outlets with the right kind of outlets to sell products which can be sold as a part of the normal journeys and flows.

Bino Pathiparampil: Got it. And in HealthCo when you say the diagnostic business, it is sourcing of the diagnostic business, right? Your back end would be AHLL?

Madhivanan B: Correct. So, we are primarily into originating business for them, the final labs, etcetera, are

Transcript of AHEL Q 2 FY26 Earnings Call Page 12 of 16

with AHLL. So, think of us as a digital arm for AHLL.

Moderator: The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: The first one on the hospital business. If you look at the EBITDA margin in this quarter has remained same despite strong ARPP growth of around 9%. Then you also suggested that the acuity mix is kind of positive. So, what is driving profitability to kind of remain same and since EBITDA margin in existing hospitals are not expanding despite the strong ARPP growth, how comfortable are we with offsetting the losses from the new units that we are expecting over the next four quarters?

Suneeta Reddy: So, with regards to the ARPP growth, we showed a 9%, which really indicates that we have really moved up in terms of complexity. To move up in terms of complexity and to ensure that we are prepared for the new hospital expansion as well as growing, getting new volumes into the old, there was considerable amounts above INR 67 crore spent on doctor hiring. I think we have created an organization structure that will make sure, that we will grow in terms of, we have improved sales and marketing, and there is certain costs attributed to that.

We have created teams that strengthen our projects division, as well as started recruiting for the new hospitals. So, we have the cost coming ahead of the opening, which is why it seems, it is at 24.6%. But going forward, we should see the benefits of all of this. And therefore, the impact of the losses in new hospitals will remain at INR 140 crore to INR 150 crore.

A. Krishnan: So also, if you look at it in a very different manner, last year same quarter, we did see an increase in spike in the margin because of the medical admissions that we spoke of. However, full year margins were at 24.2% full year. Whereas if you look at H1 of this year, we are at 24.6%. So, the way you can look at it is some, we have taken steps which is evident in terms of ensuring there is a margin flow through which happens because of the ARPP etc. We are not seeing it because of looking at it on a quarter-over-quarter basis. But when you look at it on an annualized basis, you will see that this year margins should be higher than last year reported margin.

Suneeta Reddy: And also, there is a plan to cut costs by about INR 120 crore. I believe that we have achieved about INR 60 crore of it. So clearly, this will continue to support the EBITDA margins in the range of 24.6% to 25%.

Kunal Dhamesha: And this INR 120 crore cost cutting, is it related to hospital business, and which are the areas within?

Madhu Sasidhar: So, it is a broad-based approach to managing our costs. We found opportunities in our materials management and supplies. Certainly, we are looking at how we utilize our HR more

efficiently, given that we have made huge commitments and huge investments in our digital technologies, therefore, that is driving a lot of efficiency in how we do our work. So, as we bring online new hospitals rather than the, we are actually looking at redeployment that is leading to better ef

ficiencies than how we use our HR. Beyond that, I think there will be some opportunities to reduce our ongoing costs in IT and some other costs as well.

Transcript of AHEL Q 2 FY26 Earnings Call Page 13 of 16

Moderator: The next question is from the line of Avnish Burman from Vaikarya.

Avnish Burman: My questions are on Keimed . I wanted to know that because of the GST change that happened in this quarter, how much sales did the business do, only the Keimed . How much sales loss was there?

Obul Reddy : You will see somewhere about 4% of the sales revenue coming down because of the, 12% items into 5% item, something like that. The entire 12% GST slab is removed and that consists of almost 70%. That moved to 5%, and we expect about 4%, 4.5% revenue drop .

Avnish Burman: 4% to 4.5%. Okay. And is the inventory back to normal at the customer end at the pharmacy end?

Obul Reddy : It is from day one, we have been managing that. We have planned it well and the transition was smooth that including our 700 frontend stores, we could implement this software and see the new billing is done at 7:00 AM on 22nd and everything is passed on to the customer as per the regulation .

Avnish Burman: No, my question was that in that interim period, we saw the inventory coming down at the retailer level. Is that back to normal?

Obul Reddy : That is back to normal.

Avnish Burman: Okay. And one question on the margin compression. We have seen Keimed margins getting compressed by about 40 basis points sequentially and Y-o-Y. Is there a reason for that?

Sanjiv Gupta: This is only one time . This is related to the scheme and integration related expenses, sir . And you do not see that happening in the next coming quarters. Even if those expenses are there, they will be very low. They will not be material to pull down the EBITDA percentile . But Q2 has happened because of these expenses.

Avnish Burman: Okay , thank you so much.

Moderator: The next question is from the line of Kritika Damani from Prospera Financial Solutions .

Kritika Damani: Congratulations on another strong quarter. I had a question about , like we have seen that the Healthcare Services has grown 11% in this quarter and your average revenue per patient is up 11%. So, the occupancy has eased up to 69%. So, in prior quarters, you have mentioned balance in occupancy and realization. Could you elaborate whether this quarter's average revenue per patient growth was primarily led by tariff revisions or richer case mix or structural shift in patient segments ?

Madhu Sasidhar : So, most of the tariff increases has already been realized previously. Almost all of our improvement in the average revenue per patient has come from an improvement in our case mix. And that is evident across all of our higher complexity specialties, including cardio, neurology, gastroenterology and orthopaedics . And it has also been broad based increase in ARPP primarily in our large hospitals in metro cities but across pretty much every single

Transcript of AHEL Q 2 FY26 Earnings Call Page 14 of 16

geography.

Kritika Damani: My second question is that you have highlighted that the artificial intelligence intervention in oncology, radiology and stroke care . Beyond your patient outcomes are you seeing any measurable operational efficiencies? Because how scalable are these models across your own hospitals?

Madhu Sasidhar : So thank you for that question. Yes, we are using artificial intelligence technologies very broadly. I think today, if you go on our website , there is an artificial intelligence agent that connects patients with complex medical conditions to the right doctor and al

lows them to book

appointments . That business, which is from the website grew from last year to the year by

318%, mostly due to the adoption of a very scalable AI technology that our patient consumer facing.

Internally, we have, as I said before, we have implemented command centers, digital command centers that have real-time intelligence, can see patient flows in each of the hospitals in real time. And on top of that, we are layering agentic technologies to autonomously anticipate patient issues and proactively solve them. So, I think there is tremendous potential for us to have better efficiencies through the use of AI technologies.

Kritika Damani: Thank you. That was very helpful.

Moderator: The next question is from the line of Lavanya T from UBS.

Lavanya T: Just one clarification. You mentioned that Q2, there was an increase in costs related to hiring for hospitals next quarter? So is it right to assume that this was related to the two hospitals, which are expected to open in Q3 and there will be a jump again in next quarter once the hiring for the other two hospitals to open in Q4 will happen. Is that the right way to think?

Suneeta Reddy: No. I think some of it is correct. For example, in Pune, there is a richer cost in the hiring of doctors and we are just about to do the full-blown launch in Pune. So, we do have to hire some of our doctors ahead of the opening. This will happen in Gurgaon as well. And I think that for this quarter, we have captured some of the costs. There will be a little additional cost that will come in March.

Lavanya T: Okay. the hospitals which are expected to open in Q4, there will be hiring which will happen ahead, right? So that should come in from...

Suneeta Reddy: Some hiring, yes. But it is not huge figure for this year or this quarter. So, it is not huge, but it is there.

Lavanya T: Got it. Thank you.

Moderator: The next question is from the line of Madhav from Fidelity. Please proceed.

Madhav Marda: Just wanted to check that we are at about 24.5%, 24.6% EBITDA margin in the hospital business, but we seem to have already added some of the opex for newer beds. Could you

Transcript of AHEL Q 2 FY26 Earnings Call Page 15 of 16

give some sense in terms of what the base network margins are adjusted for some of these costs that you are already building in for the newer hospitals?

And then those margins, like you said, you will report the opex for some of the newer hospitals separately, which I think, is a good idea. So, the base set of margins in your view, how do you think that could progress over the next couple of years, excluding the new beds?

A. Krishnan: So, in the base, we would like the approximate cost which is there in this quarter would be roughly around INR 10 crore in this quarter, but next quarter onwards, you will see it increase. But we will show you the split from the next quarter of our established hospitals and the new hospitals, both the revenue and the EBITDA such that it is clear for you on how it is the existing or the established is progressing, as well as the new is coming in. So that will clearly give you the perspective.

As of now, there are costs of roughly around INR 10 crore which is built in. And on the doctors side also you will see almost 5, in the doctor now. But you will see that being separated out from the next quarter. We are clearly hoping and working on ensuring that we should get the overall margins over the 25% in the next year and even higher on the established hospitals, which is at 24.6%, which is a combination of both revenue as well as costs that we spoke of.

Madhav Marda: So, the INR 15 crore cost, which is built in quarter 2, that is roughly 50 basis points, right, of our revenue, right? So basically, the base network is

above 25% in quarter 2.

A. Krishnan: So, you are right. You are right in the way that you are looking at this. But once we start splitting out from the next quarter, you will see the exact difference coming in.

Madhav Marda: And that 25.1%, which we are at in quarter 2, just that number, of course, excluding the new beds, that do you think there is a scope to expand those margins over the next one or two years?

A. Krishnan: Yes. Clearly, there is because there is here for growth. We are working on the clinical program. And clearly, we would, the internal target is to take it higher by 500 basis points.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Recently, there has been a significant hike in CGHS rates by the Central Government. So the question here is that - a) I mean, does these hikes in CGHS, I presume it will be followed by other Central Government agencies, does that change your perspective towards Government business? And two, how much impact does it really have on our business right now?

A. Krishnan: So that is a marginally better number that we have given. But from a base perspective, it does not change the views that we have been having on the business.

Moderator: We will take our last question from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Just on the CGHS rate revision, have we kind of done an exercise where we can, you can suggest what is the average rate hike that we have received or the government has given for

Transcript of AHIL Q 2 FY26 Earnings Call Page 16 of 16

the key therapies like onco, cardio etcetera.

A. Krishnan : So, the point is that I see there are certain specialties where they have increased the prices reasonably. So cardiac, onco, ortho etcetera will be reasonably better now. But the point is when you empanel yourself for CGHS, under the rule book, we have to take whichever patient they send to us or whoever comes to us. You can not deny admissions for the others. And it is not appropriate that we take that fruit unless the Government allows us to do that, then we could actually have selected some of these specialties and worked on those specialties. So, it is a bit of a difficult way of doing that. It is not something that we have been doing in the past either. So, then when you look at the overall average realization and compare it to the insurance business or even the cash tariff it is still a good 65% discount to the overall realization that we have. So that is, we do not make that kind of margins you know that.

Kunal Dhamesha: Thank you and all the best.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for the closing comments.

Suneeta Reddy: Thank you, ladies and gentlemen, for joining this call. I do want to say that the management is always available. I could see there were many more questions that needed to be asked. So please feel free to reach out to us. We are encouraged by the robust growth in revenue across all verticals. And the EBITDA we have delivered despite seasonal market factors.

Our integrated health care ecosystem spanning hospitals, diagnostics, pharmacies and digital platforms will continue to deliver balanced growth and improved asset utilization. Our footprint expansion will go on stream as planned and will add to strength to our clinical delivery, as well as to volumes and continue growth. We really appreciate your continued interest and look forward to communicating with you throughout the year. Thank you.

Moderator: On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you all for joining us and you may now disconnect your lines.

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cript and may contain transcription errors. While the transcript has been edited for clarity, the Company takes no responsibility for such errors. Efforts have been made to ensure a high level of accuracy, and any figures that may have been inadvertently mentioned have been reviewed and corrected as necessary.

Call: Feb 2026

Page 1 of 18

Apollo Hospitals Limited
Q3 FY '26 Earnings Conference Call
February 11, 2026

Moderator: Ladies and gentlemen, good day, and welcome to the Apollo Hospitals Limited Q3 FY '26 Earnings Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this call is being recorded. I now hand the conference over to Mr. Mayank Vaswani from CDR India. Thank you, and over to you.

Mayank Vaswani : Thank you, Yashashri. Good afternoon, everyone, and thank you for joining us on this call, hosted by Apollo Hospitals to discuss the financial results for the third quarter and 9 months of FY '26, which were announced yesterday.

We have with us today the senior management team represented by Mrs. Suneeta Reddy, Managing Director; Mr. A. Krishnan, Group CFO; Dr. Madhu Sasidhar, President and CEO of the Hospitals Division; Mr. Madhivanan Balakrishnan, CEO of Apollo HealthCo; Mr. Sriram Iyer, CEO of AHLL; Mr. Sanjiv Gupta, CFO of Apollo HealthCo; and Mr. Obul Reddy, CFO of the Pharmacy business.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward -looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties, which is on Slide 2 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier, and these have also been posted on the corporate website.

I would now like to turn the call over to Mrs. Suneeta Reddy for her opening remarks. Thank you, and over to you, ma'am.

Suneeta Reddy : Thank you, Mayank. Good afternoon, everyone, and thank you for joining us on today's earnings call. I trust that you have all received our earnings material that we shared yesterday. We are pleased to report a strong performance during what is typically a seasonally weak

Page 2 of 18

quarter. Importantly, we have sustained the positive momentum from the first half of the year into Q3, delivering double -digit top line growth across all three of our business verticals, Healthcare Services , Apollo HealthCo and AHLL.

On a consolidated basis, revenue grew by 17% year -on-year to INR 6,477 crore . Within this, the Healthcare Services business recorded revenue of INR 3,183 crore , reflecting a healthy 14% year -on-year growth. This growth was driven by a well -balanced mix, 5% from volume growth, 4% from case mix, and the remaining 5% from pricing.

Surgical volumes grew by 6% during the quarter, supported by our continued focus on CONGO -T specialties, cardiac, oncology, neurosciences, gastroenterology, orthopedics and transplant. These specialties remain a key growth engine for us and delivered a robust 16% year-on-year revenue growth. Group -wide occupancy stood at 67% in Q3 FY '26.

Insurance and cash patients together accounted for 83% of inpatient hospital revenues for quarter 3 FY '26, underscoring the strength and resilience of our payer mix. Average revenue per patient was INR 180,917 in Q3 FY '26 compared to INR 173,246 in Q2 FY '26, reflecting an increase in clinical intensity during the quarter.

Apollo HealthCo reported revenues of INR 2,827 crore , 20% year -on-year growth. Revenues from Apollo Health & Lifestyle increased by 20% year -on-year to INR 467 crore during the quarter.

Consolidated EBITDA for the quarter was at INR 965 crore , registering a robust growth of 27% year -on-year. Within this, Healthcare Services was at INR 719 crore , up by 18% with margins at 24.8%. Within Apollo HealthCo, the pharmacy distribution business recorded EBITDA of INR 195 crore compared to INR 159 crore last year, reflecting a 23% year -on-year increase.

Losses in the Digital business were at INR 67 crore . Cumulatively, Apollo HealthCo more than doubled EBITDA to INR 128 crore in Q3 FY '26 compared to INR 57 crore in Q3 FY '25. Cash losses in the Digital business were at INR 29 crore , the lowest in any quarter by far.

The private label and generics accounted for 15.53% of total pharmacy sales. Our digital platform, Apollo 24/7 added 2 million new users during the quarter and now serves over 46 million users. Platform GMV stood at INR 525 crore , a 28% growth over last year.

AHLL delivered an EBITDA of INR 48 crore , a strong 39% year -on-year growth with

margins improving to 10.2% from 8.8% in quarter 3 last year. With all three engines of the business performing well, evidenced by double-digit top line growth alongside accretive margin and profitability expansion, we reported a consolidated PAT of INR502 crore in Q3 FY '26, a growth of 35% year-on-year.

Turning to the 9-month performance. Consolidated revenue for 9 M FY '26 stood at INR 18,623 crore, growing 15% year-on-year, supported by balanced expansion across all three verticals.

Page 3 of 18

Healthcare Services reported revenues of INR 9,287 crore, up by 12% year-on-year, driven by continued traction in high-acuity specialties and an improving payer mix. Apollo HealthCo delivered revenues of INR 7,960 crore, registering a 19% year-on-year increase, while AHLL revenues grew 19% to INR 1,376 crore.

Consolidated EBITDA for the 9-month period stood at INR 2,758 crore, reflecting a 22% year-on-year increase and PAT grew at 34% year-on-year to INR 1,412 crore.

During the quarter, we operationalized 75 beds in our Pune facility. As we enter the next fiscal, we will be commissioning four new hospitals, one each in Hyderabad, Kolkata, Bangalore and Gurgaon, further strengthening our presence in key metropolitan markets with strong fundamentals. These facilities, along with the ramp-up in our recently commissioned Pune hospital, will approximately add 1,500 additional operating beds to our network, representing a significant step-up in capacity and

a clear runway for medium-term growth.

We expect to operationalize roughly half of this capacity in the upcoming fiscal year with the balance coming online early FY '28. This phased commissioning approach allows us to calibrate ramp-up efficiently, optimize capital deployment and drive occupancy-led operating leverage as demand scales.

Together, these additions position us well to capture growth opportunities in high-acuity care, deepen our market penetration and enhance long-term shareholder value. We have also made progress with respect to the regulatory integration process for the composite scheme of Keimed merger and demerger of Apollo HealthCo and remain well positioned to capture the full benefits of scale with the combined entity to achieve a run rate of INR 25,000 crore in combined revenues with 7% EBITDA.

Let me conclude by reiterating that Apollo's performance over the recent quarters reflects the depth, resilience and scalability of our integrated Healthcare ecosystem. Consumer interactions across all formats of care have increased and cross-format journeys are becoming more visible.

These results are a reinforcement of our patient-centric strategy. We have and will continue to invest ahead of the curve on in-hospital technologies such as robotics and the benefits of such investments are reaching the patient as evidenced by our growth in high-end surgeries. More importantly, we believe these results demonstrate the deep level of trust that our consumers place in Apollo. We value this trust and engagement and we will continue to sharpen our clinical differentiation, expand our capabilities across high-acuity specialties and strengthen our omnichannel healthcare platform to improve access, efficiency and high-quality care outcomes.

On that note, I would like to hand it over to the moderator and open the line for questions. I have Krishnan, our CFO; Dr. Madhu Sasidhar, the CEO of the Hospitals Division; Sriram Iyer, CEO of AHLL; Madhivanan, CEO of Apollo HealthCo; and Obul Reddy and Sanjiv from Apollo HealthCo with me to take all of your questions. Thank you.

Page 4 of 18

Moderator: Thank you very much. We will now begin the question-and-answer session. Anyone who wishes to ask a question may press star and one on the touchtone telephone. If you wish to remove yourself from the question queue, you may press star and two. Participants are requested to use handsets while asking a question. Ladies and gentlemen, we will wait for a moment while the question queue assembles.

We will take our first question from the line of Binay Singh from Morgan Stanley. Please go ahead.

Binay Singh: Hi team. Congratulations on a strong set of numbers across businesses. I will first start on the hospital side. It is a very busy time for you, two hospitals you have ramped up and we also have Sarjapur and Kolkata around the corner. Could you -- last time we discussed about INR150 crore cost headwind coming out of this ramp-up. Any updated thoughts on that? How much of it is already built into these numbers? How much should we expect in the coming quarters? That is the first one.

A. Krishnan: So we will come back to you by Q4 one more time, but we continue to believe that INR150 crore is a good number for now. And we have started Pune and Athena just by the last month of the -- in the previous quarter. And we are hoping to ramp both of that up over the next 2,

3 quarters well.

Currently in the embedded numbers, we have approximately INR15 crore of losses in the overall reported numbers for Pune and Athena, which is part of the deferred numbers that has been reported now. As we go into next year, we are hoping that by Q1, we should be operationalizing Hyderabad, the Calcutta facility as well as the Bangalore, which is the Belenus one.

Gurugram would be more like Q2, because it is still, because of all these issues with the environmental re

lated delays, which we could not complete our construction. There is a 2, 3 months delay. So it will be mostly in Q2.

Binay Singh : Okay. So to an extent, some of the costs of that would start to come up in the March quarter, right, all your hiring costs and all?

A. Krishnan: March or more likely April.

Binay Singh : Okay. And secondly, just on the GMV of the Digital business. We have seen a sequential drop in GMV. And in fact, our revenue to GMV ratio also went up. Could you share your thoughts on that?

A. Krishnan: Madhi?

Madhivanan B.: Sanjiv, you want to explain the reinstatement of the GMV, please?

Sanjiv Gupta : I can take that question. Thanks, Madhi. Yes, it is a good call out. See, there are two things which has happened. One is that on 31st of September, we had a very large reduction in the GST on the pharmacy and the other products, which resulted into GMV impact of roughly INR30 crore to INR35 crore a quarter.

Page 5 of 18

And secondly, we had one channel of e-commerce, which was Amazon. We were supplying to Amazon and then we stopped that business or that segment somewhere in early Q2 of this fiscal year.

And for us to compare apple -to-apple, we had to remove these two. A cumulative impact of these two would be roughly INR75 crore for Q3. These are the only two adjustments that have been done to ensure that we compare apple -to-apple between the last year figures and current year figures. Thank you.

Binay Singh : Thanks, team. I will come back in the queue.

Moderator: Thank you. Ladies and gentlemen, in order to ensure that management is able to answer queries from all participants, kindly restrict your questions to two at a time. You may join back the queue for follow-up questions. The next question is from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: Yes. Thanks for taking my question. Just extending the question on the Digital business. I see that the revenue growth here has also been low, sort of, I would not say lower, but there is been a moderation in revenue growth as well quarter-on-quarter. One, can you take us through what should be the revenue base for the digital business? What drove this moderation? And second, from a guidance perspective, do we still keep our guidance for cash EBITDA breakeven for the Digital business? I think it was supposed to be at the end of fourth quarter? Any update there?

Suneeta Reddy : Madhi?

Madhivanan B.: Yes ma'am . No. So, our revenue projections continue. So let me -- the entire focus is on trying to build a sustainable operating model. So, if you have to peel the onion a little bit, our pharmacy online business, which is our main stay of the GMV, has actually grown by 32%. There has been a little bit of pull down in some of the other areas, because of which our revenue at an overall level, GMV level, is at a flatter basis.

However, the quality of the business is improving. If I were to give you some highlights, our discount is stabilizing. Our average order value has gone up by almost INR 111 net of the GST, which has a positive impact on our unit economics. Our cost of delivery, which is one of the biggest component of our cost structure on the revenue side is also coming down. So while -- and our marketing spends, we have been extremely frugal in the way we are building the business.

But in spite of this, showing a 32% growth in our GMV and proportionate revenue is holding on. In fact, at a CM1 level, we were positive last quarter. We have made it even better in this quarter and the trajectory continues. There have been some little changes in the way we recognize the income/revenue for the hospital business, which we are recalibrating in terms of the attribution and the kind of campaigns that we are driving.

So there was a little bit of a pause which we will reactivate it again from the next quarter, but it is a full reoperating model

that we are doing. The third area on the revenue side, where we

Page 6 of 18

are facing a little bit of a mismatch, I would not say is the way our insurance business, which we started off is getting recognized. Till September, any collections, any gross return premium, which is the equivalent of revenue was coming to our commissions on a full basis. We are almost getting 80% to 85% of the total log -ins that we do. We were able to generate commission.

However, post September, given the GST change, which happened in the health insurance industry, there has been a bit of a mismatch between the business that we book and the collections that we do. That is impacting our revenue a little bit. So it is a mismatch in the sense that my revenue recognition is getting deferred over the next 12 months for the money that I am collecting.

So some of these changes is normalizing it a little bit. So we are on our course. On our -- the forecast about our ability to close it in Q4, given this insurance story, which has sort of pushed us back by around INR17-odd crore , given this mismatch of our revenue recognition, we will be able to close this loop in Q1 of FY '27.

Neha Manpuria: So, if I were to understand it correctly, you are saying that the digital -- sorry, the digital cash EBITDA breakeven is now probably pushed out by a quarter?

Madhivanan B.: By one quarter. By one quarter, yes. Because of this insurance mismatch that has happened. Otherwise, we are very much on course. That is why you see the minus INR29 crore , which, the cash EBITDA that you are speaking about, the biggest negative is coming from the INR17 crore negative from the insurance business, which has got deferred into the next year.

So we will recognize that income as we go along. And some of these revenue recognition modules that we follow in the hospital business is undergoing a change. So these are two things. So we hope to catch it up in the next quarter. But rest assured, all the primary indicators are on course.

Neha Manpuria: Sir, on the revenue front, if I were to understand it correctly, we will see this revenue plus the IP/OP that will get resumed from next quarter. Would that be a fair understanding?

Because this quarter's number is an aberration?

Madhivanan B.: Correct. Correct. Yes, we will reactivate those.

Neha Manpuria: Okay. So only thing that has changed in the insurance bit.

Madhivanan B.: Yes. That is the one which has a bigger impact. And there are some businesses wherein our revenue gets recognized at the end of the year, because some of them are back ended, that will also come into play as we go along.

Neha Manpuria: Understood. Okay. That is helpful.

Sanjiv Gupta : Just correcting one number. I think, Madhi said -- I wanted to say, INR7 crore of insurance impact into Q3, not exactly INR17 crore out of INR29 crore . Just as a correction.

Neha Manpuria: Okay. Got it. And my second question is, in the cluster numbers that we are giving, other than probably AP, Telangana and Chennai cluster, we have seen the IP volumes pretty much

Page 7 of 18

moderate in most other clusters year -on-year. So just wanting any color as to why we are seeing this trend, whether I see it in North, West or East cluster, there seems to be a moderation in the IP volume trends.

Madhu Sasidhar : Yes. So, I think you are seeing different phases of optimization. Especially in our West market, we have undertaken a lot of work to improve the quality of revenue. So I think volume by itself may not be the right indicator. We took a more holistic look into it, especially at the quality of revenue and the average revenue per patient. So especially our Navi Mumbai unit has been performing very well on that basis, especially with higher specialty care.

Neha Manpuria: All right. Okay. Got it. Thank you so much.

Moderator: Thank you. Next question is from the line of Karan Vora from Goldman Sa

chs. Please go ahead.

Karan Vora : Thank you for taking my question. My first question is with respect to the bed expansion. So I think we have mentioned other than Gurgaon, all the other greenfields or the larger units will come up by Q1. But just wanted to get a sense on like how many beds will be operationalized in Q1?

And how will the ramp up look like? I guess you will not operationalize all the beds, right?

So at a hospital level, how the ramp -up could look like over the next 6 to 9 months or even 1 year, that will be helpful for each hospital.

A. Krishnan: So, we will come back again by Q4 on this. But broadly, if you look at it, Sonapur is around -- Calcutta has 225 beds. Half of that should get operationalized in Q1. Hyderabad is 300.

Again, we think that at least 50% should get operationalized by Q1. Pune also, we have now operationalized 75%, but we think by Q1, we will add another 100 beds, which will also get operationalized. And Gurugram would be by Q2 200, 250 beds. Sarjapur, 150 beds, we are hoping that we should be able to operationaliz e 100 beds out of that.

Karan Vora : By Q1?

A. Krishnan: That is right. So roughly around 50% of these beds, right, 1,500 beds, if you look at it in these

-- we have Jubilee Hills expansion and Secunderabad which will come a bit later, which will be during the year. But of these four new hospitals, almost 40% to 50% will get operationalized by Q1.

Karan Vora : Got it. Got it. And like when we operationalize the remaining half in Q1 FY '28 or somewhere around that time, will there be any more start-up losses or fixed cost or elevations, which we need to keep in mind?

A. Krishnan: It will be there. So we will have to look at it as in a phased manner. So, we will see that Q1 should have -- so when we are saying at INR150 crore, there will be quarters which will have some higher losses also, right? There could be a quarter where there is a INR50 crore loss, which is coming in. But otherwise, we should look at overall at INR150 crore for the next year is how we would look at it.

Page 8 of 18

So it is difficult to guide now quarter-by-quarter. We will definitely, as we operationalize these, give you a broad guidance. But you can -- we have said that this is something that we are hoping that can be defrayed by the overall volume and revenue growth in the existing hospitals.

Karan Vora : Okay. Got it. And my second question is with respect to the ARPOB growth. So, I understand that we have moved away from ARPOB. But just if we back calculate ARPOB growth, it is healthy at around mid-teen levels. So just wanted to get a sense that what is driving this growth? And in the opening remarks, there was a mention of 5% price growth. But in the PPT, it is mentioned 3% price growth. So what is that mismatch, if you can just tie it down, whether it is 3% or 5%?

A. Krishnan: Sure. 3% was the tariff increase that we had taken during the year, whereas 5% is the effective price realization that we have done in this year, because there were some insurance contracts which got reset also during the year. So the tariff, what has been presented in the PPT is more the tariff increase, which was done in this year. So this is -- that is the difference between the two numbers. Your other point on ARPP, of course, we do not give ARPOB, you can compute the ARPOB, of course.

But because we do not do ARPOB because the reason is, again, we have OP, we have radiation therapy. We have other stuff also and a lot of daycare procedures also now come in. ARPP has gone up, which is a combination of, as we said, higher complexity cases that we have seen across and the CONGO-T growth and surgical growth. This is what has increased the ARPP at a broad level. Pricing realization in that has been 5%, the balance has been case mix.

Karan Vora : Okay. Got it. Just one last thing, if I can squeeze in. So Apollo 24/7 or the digital piece,

overall breakeven EBITDA breakeven, is there any target, not the cash, but even after including ESOPs?

A. Krishnan: Sanjiv, you have -- you want to give them a color of ESOPs for the next year?

Sanjiv Gupta : I think, it is better would be that when we meet again, somewhere in the Q4 earnings call, we will have a better understanding about all this as we get into AOP. But I think one point clearly that the ESOP cost maximum is getting consumed until this year. And after that, we will have a very less cost on the ESOP specifically. But overall question about including ESOP, I think, I would request just to wait for quarter. We are into the middle of annual operating exercise, planning exercise. We will be in a better position to tell next time.

Karan Vora : Okay. Got it. Thanks a lot.

Sanjiv Gupta : Thank you.

Moderator: Thank you. Next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil : Hi, good afternoon. Congrats on a great quarter. Just following up on the previous question, the price increase and realization increase that you are talking about, did that kick in Q3? Or

Page 9 of 18

was it already there in Q2 and Q1?

A. Krishnan: It was there in Q2 also.

Suneeta Reddy : But mostly in Q3.

Bino Pathiparampil : Mostly Q3. So, if I look at the kind of growth you delivered in Q1 and Q2 versus Q3, there is a big step jump. And from your competitors, we have not heard about a great season or anything in the healthcare side. So can I assume a significant part of that is coming from this realization improvement?

A. Krishnan: No. See, you have to remember that some of our competitors are in various different geographies. See, please appreciate that we are a pan-India player. If you look at the in-patient volume growth in Q2, we saw 2% volume growth, whereas in in-patient volume growth in Q3 is 4.5% versus last year. So there will be various different reasons on the way that our numbers can go versus people like who are more North centric or somewhere else

because each of the competitors, not many players are as pan -India as we are. So you have to keep that in your mind.

Of course, we have got the benefit of the price realization from Q2 onwards and which has moved into Q3. But the more important point that you should also consider is the fact that there has been a higher complexity of cases also that we are focused on in the CONGO -T specialties, and we should be able to sustain as we move forward.

Bino Pathiparampil : Got it. Second, in AHLL, we have seen quite a few number of additions in the number -- sorry, quite a few additions in the number of centers. Are these mostly on the diagnostic side?

Or is it all across?

Suneeta Reddy : Sriram?

Sriram Iyer : Yes. So most of these additions are on the diagnostics side. And we also launched two new clinics, one in Chennai and one in Hyderabad. Otherwise, most of these additions are on diagnostics.

Bino Pathiparampil : Okay. And when you say diagnostic, are these like full -fledged labs or just collection centers?

Sriram Iyer : No, these are -- obviously, we have expanded into new lab, new geographies also, but most of these are the infrastructure that you create in the existing geographies. So we opened the collection center and our company -owned outlets. So that is what you can see as expansion.

Bino Pathiparampil : Understood. Okay. And finally, an update on what is happening on the HealthCo side regarding the corporate action about demerger, et cetera. How is the progress on the Keimed side in terms of accumulation of the different pieces of business that were separate, et cetera, would be great?

Obul Reddy : We have obtained Competition Commission approval and SEBI approval. We have filed with NCLT. NCLT started -- it has listed and started the hearing. And we await for the next step.

Bino Pathiparampil : Okay. And Keimed was supposed to

buy out some of the associates, which were holding part of shares in different entities. How is that progressing? Is that all progressing on time?

Page 10 of 18

Obul Reddy : Keimed has progressing on time and Keimed has streamlined the entire subsidiary network, and they are 100% subsidiaries of Keimed, which is going to get -- I mean, Keimed is going to merge into AHLL.

Bino Pathiparampil : Great. Thank you very much.

Moderator: Thank you. Ladies and gentlemen, we request you to restrict to two questions at a time, please. You may join back the queue for follow -up questions. Next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai : Hi, Thank you for the opportunity. My first question is on your hospital business. So not specific to you, but I just want to understand your negotiation with the health insurance companies. Like how is it going? Because we heard some of your peers had some issues. So, from your hospital, either for existing as well for some of the new hospitals, how things are progressing on contracting or onboarding of health insurance?

A. Krishnan: It does not -- So we have a good relationship with all the insurance companies across. We have always been maintaining that and we have a central relationship with them as well. And as of now, basis, yes, there has been some delays in getting certain insurance approvals in some of the markets and which is why even last quarter, we saw some of that contracts getting pushed out for renewal. But with that said, I think we are on course, and we are seeing that we are fine going forward.

Damayanti Kerai : Okay. And these contracts are generally for 2 to 3 years or you are moving for some annual renewal contracts as well?

A. Krishnan: We would prefer annual, but as of now, it is still 2 years.

Damayanti Kerai : It is generally 2 years contract. And when do you start this empanelment process for the new hospitals, say four hospitals coming next year. So when do you start your negotiations to empanel the company there ?

Madhu Sasidhar : We start the negotiations much before we operationalize the hospital. To give you an example, some of the hospitals that Krishnan spoke about, some of the agreements have been in place or there has been an agreement by both parties on the terms. So we started much before operationalization, if that answers your question.

Damayanti Kerai : Yes. My second question is again on 24/7. Actually, I am not still very clear. So you mentioned the majority of changes happened on the pharmacy post GST changes. But when we look at other metrics, say doctors' consultation number or doctors and platform, diagnostic samples, all of these numbers changed compared to what we saw in the September number? So actually, if you can help us understand what all changes were there when you are booking GMV? And if you can just clarify, you mentioned INR75 crore kind of number, which would adjust in the 2Q numbers, right? Some clarification will be great.

Sanjiv Gupta : Madhi, let me just take this question. I think what we are trying to say is a very simple point that, when you compare numbers across previous year, on the overall GMV side, there are

two factors which has undergone change. One is the GST and secondly, one of the channels that we closed. So obviously, when you compare GMV to GMV, this factor has to be taken into consideration while seeing growth things. So this is one change that has happened. Now as far as -- you also checked -- I think you asked this question also that total revenue for Q3 versus Q3 FY '25 versus Q3 FY '26, you are seeing a little lesser growth or 15% growth. I think the answer there lies with the Amazon channel, which is part of the sales for Q3 FY '25, which is not part of the sales in Q3 FY '26.

And if I minus that out, you would see a growth of 32% on the overall revenue versus 15%, which is

now mathematically can be calculated. So this is just, I would say, a small correction because of the GST came in somewhere in Q2, 21st of September, as I said. So we had to change the numbers for Q3 for comparative purposes. And somewhere in April month -- somewhere in June end, we closed the Amazon channel and hence, those numbers from Q2 had to undergo change. So this is just a change in the numbers consequent to these two things to arrive at a better growth factors.

On the diagnostics and the consultation side, there is no change. The business remains as it is. Whatever commission rates are there between the entities, those related party agreements are still good to go. There is absolutely no change.

And I think you also touched upon IP/OP GMV, I think IP/OP is one -- in the last earnings call, we did -- Madhi did mention this that the new arrangement with the hospital is more of a flat fee that we get on a quarterly basis. And it is like a booking or ensuring the tech platform for the entire Apollo ecosystem.

And that is where the commercials are between both the organizations. I think this should be helpful. In case you still think that you have got some doubts, maybe separately, we can connect and then we can discuss about this.

Damayanti Kerai: Sure. So just like in terms of annual GMV, any target we are looking for, say, a month -- this quarter, we did around INR425 crore. So will this be the run rate to look ahead? Or you think we can see much better numbers going ahead? Thank you. That is my last question.

Sanjiv Gupta: You can expect a consistent growth of around, say, 30% of the GMV for this financial year.

And once we are done with the numbers, we will come back for the next year plan.

Damayanti Kerai Thank you. All the best.

Moderator: Thank you. Next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Thanks for the opportunity. Sir, on the base hospitals per se, if I exclude the new hospitals, at the metros we are already at 70% occupancy, non-metro 62%, ROCE of 31%. So, if you could just elaborate in terms of what is further scope for these hospitals to sort of drive EBITDA going forward, maybe through CONGO or patient mix or let us say the payer mix? That is my first question.

Madhu Sasidhar: Yes. Yes. So I think, even in Q3 of last year, we had reduced a little bit. You are right, the occupancy at our metro units is fairly high. There are several levers that we are depending on. One is, I think there is a little bit more opportunity on length of stay reduction through operational excellence, a lot of investment in digital technologies that is helping us with that. The second is there is still some volatility, I think, day-to-day, week-wise and seasonal volatility, and we are looking at ways in which we can minimize that volatility. And the third is, of course, a consistent focus on case mix and especially in our flagship hospitals, making sure that there is an intentional shift to high-complexity cases.

Tushar Manudhane: Any broad number, if you could share like the seasonality impact for the quarter or, let us say, for the 9 months, how much it has been?

Madhu Sasidhar: Yes. It is very hard to tell because, as you know, it varies from year-to-year, right? And it especially shows up on the medical cases. Some years, we have a bad dengue year and that skews the occupancy number. So that is the unpredictable part of our business.

But there is a lot that we can predict, which is usually on the elective and semi-elective cases, the surgical cases and the high complexity cases like solid organ transplant, where we have made a lot of commitment and a lot of investments. Transplant, as an example, this quarter compared to last quarter is about a 50% increase in revenue at the group level.

Tushar Manudhane: Got it. And just secondly, on the

the overall combined Keimed plus offline plus online, we are at a quarterly run rate of INR50 billion. And the target is to reach INR250 million annualized by Q4 FY '27, which is effectively INR60 billion, INR62 billion. If I go by earlier quarter numbers, we are sort of growing at 4%, then what could be -- while we have explained that in detail in terms of the online pharmacy or the HealthCo, but any other factors which will drive this number to INR62 billion by Q4 FY '27?

A. Krishnan: Sanjiv ?

Sanjiv Gupta: I think all the business lines, whether it is the frontend or Keimed or online, as you rightly said, online pharmacy is growing by about 30%. We are seeing a decent growth in the other business lines also. I think where we stand today at Q3 annualized number, we are roughly at say, INR20,000 crore, and we have got five more quarters to hit the run rate of about INR25,000 crore, which is roughly about 25% from now to there.

Five quarters also, I think, if we continue to see the business with a growth trajectory of about 20%, 22% annually, we should be able to meet this number easily. At this stage, we do not see any challenge coming our way with respect to hitting the top line number. Thank you.

Tushar Manudhane: Got it, sir. Thank you.

Moderator: Thank you. Next question is from the line of Lavanya from UBS. Please go ahead. Lavanya, your line is unmuted. Please go ahead with your question.

Lavanya: Yes. Thank you for the opportunity. So I just wanted to check how sustainable you see the hospital margins, given the new hospitals being operational over the next couple of quarters?

Page 13 of 18

Madhu Sasi dhar: So, I think, we will continue to be able to maintain margins. We are carefully balancing the EBITDA deterioration in our new hospitals with how we are managing our existing hospitals.

Also, I think for the new hospitals, we are very, very focused on quickly ramping up to profitability by both making sure that our recruitment and our human capital costs are aligned with the occupancy numbers.

Lavanya: Just a question in this. Actually, here, even in this quarter, we have some new operational beds, but still if I see consol employee cost, it has been down on a sequential basis. Any specific reason there?

A. Krishnan: So in Q2, there were two one-offs which are there also. So if you look at Q2, there was a sick leave encashment provision that was required under the accounting standard that was a INR12 crore number. And there was also an additional cost of PLVP or performance-linked variable pay that we have in Q2, because July is when we do the -- we do the increment. So this was there in Q2, and that is not there in Q3. Otherwise, it is aligned with the numbers.

Lavanya: Okay. Okay. Got that. Just on the physical pharmacy, how do you see growth in terms of on sustainable basis, like store addition plus same-store growth overall, like how do you see it 18%, 20%? Or what is the level that you expect in terms of physical pharmacy growth?

Obul Reddy : Now, we are currently at about 20%, 20.5% on the total network. On the same-store growth, we expect about 18% and store additions will continue to be in the range of 600 per annum.

Lavanya: Okay. We should maintain this kind of run rate going ahead in near to medium term? Is that the right assumption?

Obul Reddy : Yes. We are confident of.

Lavanya: And 18% of the same-store, how should one should look at it? Like what should drive 18% to 20%?

Obul Reddy : 16% same-store growth as against the 20.5% overall growth.

Lavanya: Okay. Key drivers for the 16% is increasing private label or how should one see it?

Obul Reddy : Private label then, we even doing the refresh store, changing the model, changing the inventory, particularly with the -- dynamically . All those things add s to that growth.

Lavanya: Okay. Okay. Got it. Got it. Thank you. Thank you so much. All the best.

Moderator

r: Thank you. Next question is from Vivek Agrawal from Citigroup. Please go ahead.

Vivek Agrawal: Hi. Thanks for the opportunity. So one question on hospital ARPP growth. So 10% growth in 9 months looks quite impressive. So I just want to understand how sustainable it is. So can you maintain this 10% kind of growth? Or there is a possibility that it can come down? Thank you.

A. Krishnan: So see, we have been always guiding saying that we would like to achieve more volume growth, and we would continue to focus on getting the volume growth to a higher number.

And then the combination of volume plus the case mix focus, which Dr. Madhu already said.

Page 14 of 18

So that is the mix that you would get to.

So maybe it is -- this is a very dynamic number in the industry, right? As we focus on a certain clinical programs in a quarter and then depending on how the clinical programs ramp up, you will see the ARPPs come up accordingly. So we would like to believe that when it is a 12%, 13% organic growth that we look at of 14%, we would like to be half on the volume and the balance half on combination of case mix and pricing.

Vivek Agrawal: Just one question again on margin trajectory next year as you are guiding for INR150 crore kind of loss in the new units. So just want to understand, do you have levers in the existing network that can mitigate the impact of losses in the new units? Or are you seeing a significant dip as far as the margins or the hospital business margins cumul atively in the next year?

Suneeta Reddy: No, I think the lever that we have is asset utilization. So as we look at our assets and occupancy and even in spite of bringing down the ALOS, we have a headroom for lifting volume and asset utilization by another 8%. We will focus on this as we go forward into the New Year. The second, of course, we have not done any significant cost cutting, which we hope will bring us another 80-100 basis points. With this, we hope to minimize the losses coming from new hospitals.

Vivek Agrawal: Understood, madam. So just one more thing. What kind of a margin expansion that is possible, let us say, in the existing business?

A. Krishnan: At least 100 basis points is the margin expansion, which is possible in the existing business next year.

Vivek Agrawal: Thank you. Just last question from my side. It is on talent retention. So as we are seeing that hospitals in India are expanding capacities, it looks like that Apollo can become a hunting ground for big doctors. And very recently, we have seen one of the star oncologists in Delhi has been poached by one of your peers. So just trying to understand how you are tackling this issue.

Suneeta Reddy: Well, I think the reverse is also happening. So I think we should be aware that Apollo will continue to attract the best talent. Because, number one, I think we have created a platform that invests not only in technology, but in terms of reach, in terms of market and market share, we will continue to lead with market share. And I think this is what doctors want. Our systems are strongly embedded so that clinical outcomes at Apollo continue to be the best-in-class. And in terms of innovation research and collaborations with other hospitals, this will continue to drive very high-end procedures. This is -- I think this is what doctors would like to see. Madhu, you want to add to that?

Madhu Sasidhar: No, I think, Ms. Suneeta is absolutely right. And I will just share our experience in recruiting in the new markets that we have opened hospitals in like Pune or existing markets where we have new hospitals is that we have had a very good brand recognition and brand affinity with the doctors in the community that has made it easy for us to recruit.

Page 15 of 18

I think that you will see the sporadic cases of doctors leaving and sometimes it is for personal reasons that has nothing to do with the hospitals that

at we operate in. So I do not see that in the next year as being a problem, either retention or recruitment.

Moderator: Next question is from the line of Madhu Marda from FIL.

Vivek Agrawal: Thank you. That is from my side.

Moderator: Thank you. Next question is from the line of Madhav Marda from FIL. Please go ahead.

Madhav Marda: Hi, good afternoon. Thank you so much for your time once again. Sir, just on the margins again, if you look at our reported margins, 24.8%. I think you said INR15 crore of cost is already there for the new units. So base network margin seem to be 25.3% approx in quarter 3. You are saying that, that 25.3% has scope to go up by 100 basis points. So base network can be above 26% next year. Is that how we should read it?

A. Krishnan: Yes.

Madhav Marda: Okay. And then so basically, the impact of the losses from the new unit, the INR150 crore should come on this 26%, right? Basically on that, we should assume the drag?

A. Krishnan: That is correct.

Madhav Marda: Okay. Understood. Okay. And sir, just a second question is on the hospital business growth. Given the time line for the expansions that we have of the new beds coming in, how should we think about hospital business revenue growth next year on the base network? And then how much revenue can we add from the newer beds, which are coming in?

A. Krishnan: So we would not want to guide specifically around that, right, because it is forward-looking. So as you know that we are looking at -- we continue to look at seeing how we can at least be at the 12%, 13%, 14% growth on the existing hospitals, but we will see -- how the year starts and how we progress. And then we should -- the additional beds we should clearly add another 3%, 4%.

Madhav Marda: Understood. Thank you very much.

Moderator: Thank you. Next question is from the line of Avnish Burman from Vaikarya Change LLP. Please go ahead.

Avnish Burman: Yes. Hi. Good afternoon. Thanks for taking my question. I just have a couple of questions on Keimed. We saw some good margin expansion this quarter on a Q-o-Q and a Y-o-Y basis.

Can you just articulate what were the key reasons for that?

Suneeta Reddy: Sanjiv?

Sanjiv Gupta: So I think what you saw was the aberration in Q2 and as well as in Q1, where the overall EBITDA percentage was low because of the restructuring and the necessary legal cost associated with them. I think -- so now those are removed. And this is what we discussed in the last earnings call also that in Q3, you would see we will be coming back to operation of 3.1 upwards and 3.3% is what we have in Q3, but it is more as an aberration that got cleared

Page 16 of 18

in Q3 then plus the business efficiencies.

Avnish Burman: Okay. And because of the GST change that happened in September, did we see some kind of revenue push out from the 2Q to 3Q because I am guessing the retailers would be reducing inventories in 2Q and then building again in Q3. Does that happen? Or on a quarter-wise basis, it was just normal Q2 and Q3 operations in revenue?

Sanjiv Gupta: No, we did not see any such impact.

Obul Reddy: We have seen very good improvement in the FMCG and pharma slightly plus or minus sales at the same level, but FMCG consumption is very good, and we have seen good growth on that.

Avnish Burman: Okay, understood. Thanks. I will get back in the queue.

Moderator: Thank you. Next question is from the line of Kunal Dhamesha from Macquarie. Please go ahead.

Kunal Dhamesha: Sir, just one on the potential losses from the new units. You suggested that the first month from just two hospital is around INR15 crore, and we are guiding for a full year losses of INR150 crore. So, is it more conservative number because the annualized losses itself would be more like INR180 crore. I am sure it would improve in the existing units. But the 150 number, is it more conservative on the side, given we are also opening a greenfield in

Gurugram, which would definitely take some more time versus the brownfield that we are doing.

Gurugram, which would definitely take some more time versus the brownfield that we are doing.

Moderator: Kunal, can you mute your line please. There is some background disturbance.

A. Krishnan: See, we have two quarters of Pune to ramp up from here on before some of the other hospitals come on stream also. So that is one that should kind of benefit us as we move into the next year. And then you will start see some of the other hospitals coming in. And so it is going to be like three quarters of next year when you will be seeing the other new hospitals coming in. So that is what we are looking at.

And -- but I think broadly, as of now, we would like to keep it at INR150 crore. That is what we would like to keep it for now. And this number of INR15 crore is really for 3 months of Pune in terms of costs of Athena. The revenues have more been in the last 1 month or 1, 1.5 months, but the cost has been for 3 months. So you should -- so that is the way you should look at it. So now we will go with INR150 crore. We will see during the next year.

Kunal Dhamesha: Sure, sir. And second one on the GMV growth that we guided, we suggested that we would be ending the year with 30% GMV growth. So that means Q4 would be 30% or we guided for the 20%, 30% for full year FY '26?

Suneeta Reddy: Madhi.

Madhivanan B.: Yes. This is on a full year FY basis. We will obtain our 30% to 35% growth on the GMV basis. In this quarter, we remove around 4% to 5%.

Kunal Dhamesha: And sir, for 9 months, this number looks much lower, right, in terms of -- so you are adjusting

Page 17 of 18

the base for the GST impact and the Amazon impact -- for this 39%?

Sanjiv Gupta: Correct. Correct. Because the Amazon business, the reason why we walked out was because it was a negative business for us. So while it has impacted the GMV on the revenue side, it is been much more beneficial. So that is the new normal now.

Kunal Dhamesha: Yes. What was the full year number for Amazon last year for us to adjust the base?

A. Krishnan: Sanjiv, can you give the exact number, please?

Sanjiv Gupta: It was roughly INR160 crore.

Kunal Dhamesha: INR160 crore on a INR3,000 crore of GMV, right? So, yes, roughly, 5%.

Sanjiv Gupta: I think another important point that you need to keep in mind is that the -- what we are talking here about is the entire platform GMV to grow year-on-year, this year growth as well as for the next year. And when we are defining platform GMV, while obviously, we discussed about GST and Amazon impact.

In this platform GMV, we are not adding IP/OP GMV. And IP/OP GMV is -- and that is the reason you would see in the earnings call presentations also, we have highlighted this as separately. Because this is this -- in the earlier year, this used to be a variable pay model. From this year onwards, it has converted into more of a fixed fees to us, supporting the technology side of expenses that we are incurring for the group as a whole.

So I think going forward, our guidance would continue to be excluding IP/OP, what is the growth that we will be looking at. And I think Madhi in one of the questions did say that next year, you can expect in the range of about 30%. This year also, if I just look at some numbers, we should be in the range of about 28% growth for the full year versus previous year adjusted for the reasons that we just discussed.

Kunal Dhamesha: Basically IP/OP, GST and GMV. These are the three impacts that should address right?

Sanjiv Gupta: Perfect.

Kunal Dhamesha: Thank you.

Moderator: Thank you. Next question is from the line of Ronak Agarwal from Ithought PMS.

Ronak Agarwal: Yes, thank you for the question, madam. So, I just want a broad overview for the hospital. So 1,500 beds will be coming in the next year out of which 50% will be operationalized. So what is the 50%, 60% -- what is the time period which we are looking for, let us

say,

occupancy of 60%, 70%?

A. Krishnan: On the overall -- all the beds or half of the beds?

Suneeta Reddy: So we will open 750 beds next year -- sorry, in the coming year. And post that, another 750 beds. In 2 years, we should be breakeven on 1,300 beds that we are talking about.

Ronak Agarwal: So I am asking, like, let us say, we are opening 750 beds. So what is the occupancy rate we are looking in the first year itself?

Page 18 of 18

Suneeta Reddy: It will be around 40%.

Ronak Agarwal: Okay. Okay, madam. Thank you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand the conference over to management for closing comments. Over to you.

Suneeta Reddy: Thank you all for your presence. I am sure that in the upcoming quarters, we will continue to remain focused on phase d capacity operationalization and the opening up of new beds in Healthcare Services and a continued ramp -up of revenue and profitability in AHL and AHLL. Disciplined execution leading to volume and value growth, sustained investments in clinical excellence and technology and market share gains across key markets will help us to sustain momentum over the medium term.

We are committed to delivering consistent performance while creating enduring value for our patients, consumers, partners and all of our shareholders. So, thank you all for your support.

Moderator: Thank you, members of the management team. On behalf of Apollo Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.